CLINICO-SOCIAL CASE PRESENTATION

Nisarga Kini

General Information

- Name of the patient: Mrs. Pavithra
- Gender: Female
- Age: 24 years
- Education: Second PUC
- Occupation: Housewife
- Address: Kengeri Upanagara, Bangalore
- Religion: Hindu
- Date of Admission:21/1/2020
- Date of Examination:23/1/2020
- LMP: 3/5/2019
- EDD: 10/2/2020
- Gestational age: 38 weeks

Family Details

- Type of family: Joint Family
- Composition of Family:
  - No of members: 5
  - Head of the family (HOF): Manoj Kumar

Sr no.	Name	Relation to HOF	Age	Sex	Education	Marital status	Occupation	Income	Remarks
1	Pavithra	wife	24	F	Second PUC	married	housewife	-	-
2	Manoj Kumar	HOF	30	M	SSLC	married	carpentar	15,000	-
3	B.Malavathi	mother	55	F	8th grade	widowed	homemaker	-	-
4	Dhanush	son	36 mon	M	-	-	-	-	Immunized till date
5	Padmavathi	Grand mother	77	F	-	widowed	homemaker	-	hypertensive

- Total monthly income of the family from all sources: 15,000
- Per capita income: 3000
- Above poverty line
- Socio- economic status: lower middle (9) by modified kuppuswamy scale.
- Health services:- Anganwadi, PHC
- Social institutions:- School, Bank
- Card holder:- Thayi card
- Widow pension – Rs 600 for B. Malavathi
  Rs 1000 for Padmavathi

Medical History

- Mrs. Pavithra presented with 9 months of amenorrhoea and is admitted for safe confinement.

Obstetric History

- Obstetric Score:- $G_2P_1L_1$
- Married for 31/2 years
- Non consanguinous marrriage

History of present pregnancy

- Spontaneous conception, pregnancy diagnosed by UPT at 1 month and was confirmed at hospital.

1st Trimester :

- History of nausea and vomiting
- History of folic acid intake

- No history of fever with rashes
- No history of exposure to radiation or drug intake
- No history of bleeding pv
- Dating scan was done at 6 weeks and revealed a singleton pregnancy
- NT scan was done at 11 weeks the results were found to be normal
- Routine investigation of blood and urine: normal

2nd Trimester

- Quickening at 16 weeks
- one dose of TT injection (16 weeks) was given
- History of intake of iron and calcium
- No history of pedal edema, epigastric pain, blurring of vision, headache and vomiting

- No history of excessive weight gain, excessive thirst, hunger, frequent micturation, fatiguability
- No history of leaking or bleeding pv
- Anomaly scan was done at 20 weeks and the results were found to be normal.
- Routine investigation of blood and urine: normal

3rd Trimester

- Continued to perceive fetal movements
- Continued to take iron and calcium tablets
- No history of pedal edema, epigastric pain, blurring of vision, headache and vomiting
- No history of excessive weight gain, excessive thirst, hunger, frequent micturation, fatiguability
- No history of leaking or bleeding pv
- Interval growth scan was done and the report was normal
- Routine investigation of blood and urine: normal

Menstrual History

- Menarche at age 14 years
- Regular cycle of 28 days
- Duration of menses:- 3-4 days
- No of pads used per day: 2-3
- No history of passage of clots and dysmenorhoea
- LMP: 3/5/2019
- EDD: 10/2/2020



- Booked case: Yes
- Registered case: yes

Past History

- No history of diabetes mellitus, hypetension, thyroid disorders, epilepsy
- No history of any drug intake

Details of ANC care

- No. of ANC visits:-4
- IFA tablets : taken
- History of TT immunization: given(1 dose)



- History of previous pregnancy

- 3months after marriage, spontaneous conception, confirmed by UPT after 1 month
- History of polyhydramnios in first pregnancy, full term vaginal delivery, male 3.1 kg, cried immediately.
- Breast fed for one year, complementary feeds were introduced during 6th month
- Child is immunized till date
- Inter pregnancy interval – 1 year 11 months
- No use of contraceptives in between



Family History

- Nothing significant

Personal History

- Appetite: good
- Sleep: undisturbed
- Bowel and Bladder movements: Normal
- No addictive habits

Personal Hygiene

- good



Diet History

- 24 hour recall method

S.No	Time	Food Items	Quantity	Calories (kcal)	Protein (g/dl)
1.	8am	Idli	4	300	4
2.		sambhaar	1 cup	110	3
3.		coffee	1 cup	110	3.2
4.	11am	banana	1(medium)	90	0.9
5.	1pm	Plain rice	2 cup	340	7
6.		dhal	1 cup	350	21
7.	4pm	coffee	1cup	110	3.2
8.		apple	1	65	0.2
9.	8pm	Plain rice	1 cup	170	3.5
10.		dhal	1 cup	350	21

- Total calories:-1995kcal Total proteins:-67g
- Expected:- 2250kcal Expected:- 78g
- Deficient in calories: 255kcal Deficient in proteins: 11g

Family and Environmental Study

- Type of house:- Pucca, Rented house
- Overcrowding:- absent
- Ventilation:- adequate
- Lighting:- natural light – adequate
  artificial light - adequate
- Mosquito breading areas – absent
- Fly breeding areas – absent
- Rodents in the house – absent
- Pets in the house – absent



Kitchen – separate

- Storage of cooked food:- copper vessels (covered)
- Storage of uncooked food:- shelves(kept covered)
- Fuel used:- LPG
- Chimney- present
- Washing areas for utensils - basin
- Drainage – closed

Water supply- public

- Continuous through taps
- Source of drinking water: Kaveri
- Storage of drinking water:- container(covered)

- Household purification of water:- Boiling
- Water for other purposes: Muncipality provided water

Refuse disposal

- Stored in a covered box in the house
- Disposed once in two days in a common dustbin in the locality which is later cleared by muncipal authority

Latrine

- Own – RCA latrine, connected to muncipal sewerage system
- Tap – continuous water supply
- Hand washing practices – with soap

Social History

- Food taboos:- Did not eat papaya, pineapple during first pregnancy and following the same now
- No other cultural practices
- Knowledge - good
- Attitude - good
- Practice - good

General Physical Examination

- Her consent was taken and she was cooperative and oriented to time, space and person
- Examination was done under adequate exposure to light
- Patient was of moderate built
- BMI :- 23 pre-pregnancy weight:-60kg
  height: 160cm

Vitals

RR: 18 breaths/min **Temp:** 98˚ F **SPO_2**: 98%

Pulse: 77 beats per minute, regular rhythm, normal in volume and character, no radio- radio delay, other pulses felt and are normal

BP: 120/90 mm of hg measured in the right arm, sitting position

- Pallor: not seen, Icterus: not seen, Clubbing: not seen, Cyanosis: Not seen, Lymphadenopathy: not present, Edema: Not present
- Breast: normal
- Thyroid: normal
- Spine: normal

Systemic Examination

- CVS: S1 and S2 heard, no murmurs
- RS: Normal Vesicular Breath Sounds, no added sounds
- CNS: no focal neurological deficit

Per Abdomen Examination

- **Inspection**
- Shape of abdomen: globular, longitudinally distended
- Umbilicus: central and everted
- Flanks full
- Linea nigra: seen
- Stria gravidarum: seen
- No pulsations, scars or engorged veins seen
- Hernial orifices: intact



Palpation

- Fundal height: 36 weeks
- Symphsio-fundal length: 36cm
- Abdominal girth at umbilicus: 95cm

Leopold Maneuvers:

- Fundal grip:- Soft, broad, not balotable – suggestive of breach
- Lateral grip:- Uniformly curved structure on left side suggestive of spine

  knob like structures on right side suggestive of limb buds
- Pawlick's grip:- approximation of fingers suggest hard globular balotable structure- head

- Pelvic grip:- fingers are converging, head not engaged

Auscultation

- Fetal heart sound heard on left side
- FHR:- 144 beats/min



Clinico-Social Diagnoses

- 24 year old expecting mother named Pavithra with an obstetric score G2P1L1 and gestational age of 38 weeks with cephalic presentation is hailing from kengeri, studied till second PU belonging to a joint family of socio-economic status lower middle according to modified kuppuswami scale. She is a Thayi card holder and is admitted for safe confinement. She is living in good environmental conditions with proper housing, lighting and ventilation. She has good social support and her knowledge, attitude and practice is good.

Management – Individual level

- Dietary Advice
- Personal Hygiene
- Care of baby
- Breast feeding practices
- Family planning methods
- Avoidance of drugs



Family level

- Moral and social support
- Rest to mother
- Providing the mother food rich in calories
- They should encourage immunization of child



Commnity level

- Availing schemes like Thayi card, Ayushman Bharat, JSSK, NSSK, RMNCH+A, JSY, RBSK, ICDS

THANK YOU